Temper Tantrums

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Introduction

- ☑ Temper tantrums are brief episodes of unpleasant, undesirable, disruptive behaviors or emotional outbursts that are extreme and usually disproportionate to the situation.
- ☑ These are usually seen between the age groups of 18 months and 4 years and signify the appearance of autonomy while simultaneous seeking of parental attention.
- ☑ Although the majority of temper tantrums in toddlers are typical and part of normal behavior, atypical tantrums can be a presenting feature of behavioral and developmental disorders, e.g., autism.

Epidemiology

- ☑ Occurs maximum in toddlers and decreases as age increases.
- ☑ Only 10% of 4–5 year olds have temper tantrums as compared to 85–90% of 18–36 month olds.
- ☑ Children with speech delay, behavioral disorders and developmental disorders have more frequent and aggressive tantrums.

Etiology

- ☑ Commonly occur as a response to unfulfilled demands, frustration, anger or attention seeking behavior.
- ☑ The common physiological triggers are hunger, illness, fatigue, fear or overstimulating environment.
- ☑ Psychosocial factors implicated are inconsistent parenting, failure to set limits, use of corporal punishment, maternal depression, and low socioeconomic status.

Clinical Features

- ☑ Episodes of crying, screaming, going limp, flailing, hitting, throwing items, pushing, or biting.
- ☑ Sometimes may lead to breath-holding spells
- ☑ Usually occur once a day, lasting for approximately 1–3 minutes
- ☑ *In atypical/severe cases:* It may occur >5 times/ day and/or lasts for >15 minutes
- ☑ Normalization of mood and behavior in between the episodes
- ☑ Severity, frequency, and length of the events decrease with increasing age with the acquisition of skills to identify emotions, communicate feelings and implement positive behaviors to manage negative feelings or emotions.

Evaluation

A thorough clinical history including development details and physical examination is mandatory for comprehensive evaluation of temper tantrum **(Table 1)**.

TABLE 1: Physical examination and detailed laboratory tests.

Examination	***Laboratory tests***
☑ Physical examination is normal in most of the cases, however, neurocutaneous stigmata or dysmorphic facies may indicate need for further evaluation ☑ Vision and hearing impairment should always be ruled out as they can exacerbate tantrum behaviors ☑ Neurodevelopmental disorders, e.g., autism should be ruled out ☑ The Child Behavior Checklist and the Preschool Age Psychiatric Assessment may be used in children aged 2–5 years to assess associated behavioral/psychiatric issues	☑ No tests are required in most cases, however, iron deficiency anemia and lead toxicity may aggravate tantrums and breath holding spells ☑ A complete blood count and testing for lead levels may be required in some cases

Management

Usually nonpharmacological treatment suffice for most of the cases. However, pharmacological treatment may be required when tantrums are associated with severe aggression **(Table 2)**.

TABLE 2: Management of tantrums.

Nonpharmacological	***Pharmacological***
The acronym **RIDD** can help parents and caregivers handle a typical tantrum ☑ **R**emain calm, state firmly in neutral tone ☑ **I**gnore the tantrum ☑ **D**istract the child. Leave the place taking the child till tantrum stop ☑ **D**o not give into unreasonable demands and unnecessary physical punishments which can increase undesirable behavior *Parent-child interaction therapy (PC.IT) is one-on-one therapy for parents to improve communication with child and tackle tantrums*	☑ Iron supplementation may be helpful in cases of breath holding spells with anemia ☑ Low dose antipsychotic agents, e.g., risperidone or aripiprazole for few weeks as an adjuvant to nonpharmacological management may be helpful to decrease severity especially in children with associated developmental or psychiatric disorders. *(This is to be done only by an expert after referral as stated later)*

When to Refer to an Expert?

- ☑ Tantrum episode lasting for >15 minutes.
- ☑ Very severe tantrum associated with aggression towards others and/or self-injurious behavior.
- ☑ Associated neurodevelopmental disorder or a psychiatric condition.

Further Reading

- ☑ Belden AC, Thomson NR, Luby JL. Temper tantrums in healthy versus depressed and disruptive preschoolers: defining tantrum behaviours associated with clinical problems. J Pediatr. 2008;152(1):117-22.
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- ☑ Potegal M, Davidson RJ. Temper tantrums in young children: behavioural composition. J Dev Behav Pediatr. 2003;24(3):140-7.
- ☑ US Preventive Services Task Force, Curry SJ, Krist AH, Owens DK, Barry MJ, Cabana M, Caughey AB, et al. Screening for Elevated Blood Lead Levels in Children and Pregnant Women: US Preventive Services Task Force Recommendation Statement. JAMA. 2019;321(15):1502-9.

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Lead Author
S Balasubramanian

Co-Authors
Lakshan Raj

Under the Auspices of the IAP Action Plan 2022

Remesh Kumar R
IAP President 2022

Upendra Kinjawadekar
IAP President-Elect 2022

Piyush Gupta
IAP President 2021

Vineet Saxena
IAP HSG 2022–2023

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IAP Standard Treatment Guidelines Committee

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IAP Coordinator

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